- Anuria (absence of urine production) develops as the disease progresses.

Dietary Management

- Bed rest and antibiotic drug therapy along with optimum nutritional support are emphasized.
- Salt usually is not restricted unless oedema, hypertension, oliguria is present. Fluid intake is adjusted to output.

Energy

Carbohydrates should be given liberally to provide sufficient energy and prevent tissue protein breakdown.

Protein

Improper functioning of the nephrons leads to the inability to filter urea nitrogen, a product of protein breakdown which is usually expressed as **blood urea nitrogen (BUN).** If the blood urea nitrogen is elevated and urine output is decreased, dietary protein is restricted. High quality animal proteins such as meat, eggs are preferred to vegetable proteins to ensure maximum utilization.

Intake between 0.5-1.5 g/kg body weight is recommended for children. An protein intake may be held at 0.75 to 1 g/kg body weight when BUN levels are normal.

Fluid

Fluid intake is restricted according to urine output. If restriction is not indicated fluids can be consumed as desired.

Sodium

Sodium is restricted to 500-1000mg/day if oedema or hypertension is present, but may be increased to 2-3g/day as the function resumes.



Potassium

As kidneys lose their ability to clear potassium, a build up results in the body. This can cause irregular heart beats, hence potassium intake is restricted. Most of the fruit and vegetables contain potassium which can be reduced by double leaching (cooking them in excess water and discarding the water twice).

Case Study

Ramesh is 60 year old man having oedema. He is an hypertensive who complains of poor appetite and low urine output.

1. What is the diagnosis?
2. What is the sodium requirement?
3. What advice would you give him on the protein intake?

1.9.2 Nephrosis

Nephrosis results from the damage to the tissues of the nephron both the glomerulus and tubule. This disrupts both the filtration and reabsorption functions which result in large amounts of protein to pass into the tubules causing damage to it. Nephrosis is caused by progressive glomerulonephritis, diabetes, drugs, heavy metals or toxic insect venoms.

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